Margaret Rivera

78 years · female · fictional demo patient

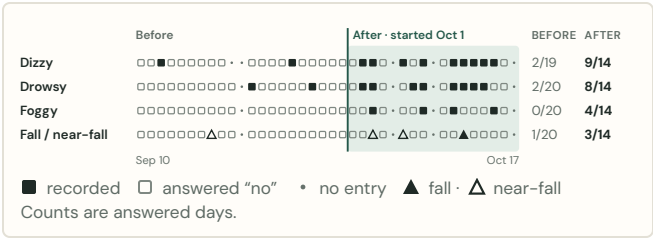
PERIOD Sep 18 – Oct 17, 2026 (30 days) | PREPARED BY Alex (daughter) | DATE October 17, 2026

Conditions (family-reported): high blood pressure, type 2 diabetes, bladder leakage (urinary incontinence), insomnia, osteoarthritis

1 WHY WE'RE COMING IN

Alex (daughter) wrote on Oct 10: "Mom seems foggier in the mornings since urgent care started the new sleeping pill. She almost fell in the bathroom on Tuesday."

2 WHAT WE'VE NOTICED



- Dizziness was recorded on 9 of 14 answered days from Oct 1, when Lorazepam (Ativan) was started, through Oct 17, compared with 2 of 19 answered days in the 21 days before.
- Daytime drowsiness was recorded on 8 of 14 answered days from Oct 1, when Lorazepam (Ativan) was started, through Oct 17, compared with 2 of 20 answered days in the 21 days before.
- Confusion or memory fog was recorded on 4 of 14 answered days from Oct 1, when Lorazepam (Ativan) was started, through Oct 17, compared with 0 of 20 answered days in the 21 days before.
- Also recorded around the time Lorazepam (Ativan) was started: sleep quality ratings averaged 3.4 out of 5 on 14 answered days after, compared with 2.6 out of 5 on 20 answered days before.
- Around the start of Lorazepam (Ativan) on Oct 1, check-ins were recorded on 20 of 21 days before (Sep 10 through Sep 30) and on 14 of 17 days after (Oct 1 through Oct 17).
- From Oct 1, when Lorazepam (Ativan) was started, through Oct 17: 1 fall (Oct 12) and 2 near-falls (Oct 3, Oct 6) were recorded on 14 answered days, compared with 1 near-fall (Sep 17) on 20 answered days in the 21 days before.

3 MEDICATION REVIEW PROMPTS

- Lorazepam (Ativan): benzodiazepine. AGS Beers 2023 lists Lorazepam (Ativan) as a medicine older adults should generally avoid. (AGS Beers Criteria® 2023)
- Tylenol PM (OTC): sedating (first-generation) antihistamine. AGS Beers 2023 lists the diphenhydramine in Tylenol PM (OTC) as a medicine older adults should generally avoid. (AGS Beers Criteria® 2023)
- Anticholinergic burden score (ACB): 7. Margaret's medicines add up to 7 on the Anticholinergic Cognitive Burden (ACB) scale, a published list of medicines with anticholinergic effects: the diphenhydramine in Tylenol PM (OTC) 3, Oxybutynin 3 and Furosemide (Lasix) 1. (ACB scale, 2012 update)
- Medicines linked to falls, with falls or dizziness recorded (5). 5 of Margaret's medicines are on published lists of medicines linked to falls in older adults (CDC STEADI-Rx and STOPPFall 2021): Lorazepam (Ativan), Furosemide (Lasix), Amlodipine, Tylenol PM (OTC) and Oxybutynin. (CDC STEADI-Rx; STOPPFall 2021)
- Possible prescribing cascade: Amlodipine → ankle swelling → Furosemide (Lasix). Furosemide (Lasix) was started on Jul 14, 45 days after Amlodipine, for "ankle swelling". (Savage 2020)

4 OUR QUESTIONS

1. Could Lorazepam (Ativan) be contributing to the dizziness, daytime drowsiness, confusion or memory fog, falls and near-falls recorded in the diary since Oct 1? Is Lorazepam (Ativan) still the best option for sleep or anxiety, and are there alternatives with less effect on balance and memory?
2. Could Tylenol PM (OTC) be contributing to the dizziness, daytime drowsiness, confusion or memory fog, falls and near-falls recorded in the diary in the last 30 days? Is the sedating antihistamine in Tylenol PM (OTC) still needed, or is there a gentler option for sleep or allergies?
3. Margaret's anticholinergic burden score (ACB) adds up to 7. Could any of these medicines be reduced or replaced with one that has less anticholinergic effect?
4. With 1 fall and 2 near-falls on 26 answered days, and dizziness on 10 of 25 answered days in the last 30 days, could any of these medicines be contributing, and could the list be reviewed for fall risk?

5 CURRENT MEDICATION LIST (8)

MEDICINE · HOW TAKEN	FOR	PRESCRIBER	STARTED	REVIEW FLAGS
Amlodipine · 10 mg once daily	Blood pressure	Dr. Okafor (cardiology)	May 30	Fall risk · **Cascade?**
Furosemide (Lasix) · 20 mg once daily	Ankle swelling	Dr. Lee (primary care)	Jul 14	**Beers caution** · **ACB 1** · Fall risk · **Cascade?**
Metformin · 500 mg twice daily	Diabetes	Dr. Lee (primary care)	Apr 30, 2024	—
Atorvastatin · 20 mg nightly	Cholesterol	Dr. Lee (primary care)	Apr 30, 2024	—
Omeprazole · 20 mg once daily	Heartburn	Dr. Lee (primary care)	Sep 12, 2025	Beers avoid
Oxybutynin · 5 mg twice daily	Bladder	Dr. Shah (urology)	Mar 21	**Beers condition + combo** · **ACB 3** · Fall risk
Tylenol PM (OTC) (acetaminophen + diphenhydramine) · 2 caplets at bedtime as needed	Sleep	Self-selected (over the counter)	Apr 20	Beers avoid + condition + combo · **ACB 3** · Fall risk
Lorazepam (Ativan) NEW · 0.5 mg at bedtime	Anxiety / sleep	Urgent care	Oct 1	Beers avoid + condition · Fall risk

Flags: AGS Beers Criteria® 2023 · ACB = anticholinergic burden (1–3) · fall-risk lists (STEADI-Rx, STOPPFall) · cascade = possible prescribing cascade. Allergies are not recorded.

6 DIARY COVERAGE & LIMITATIONS

Check-ins were recorded on 26 of 30 days from Sep 18 through Oct 17; days without a check-in are left out of every count.

Doses taken are not logged, so as-needed medicines cannot be separated.

Recorded symptoms can change for many reasons, including chance and expectation; this record does not show cause.

This review checks published criteria (AGS Beers 2023, ACB scale, fall-risk medicines, known prescribing cascades). It does not check doses, kidney function unless provided, allergies, or every interaction. No flag is not an all-clear for any medicine.

Prepared by family from home records; not a clinical assessment.

Don't stop or change any medicine without talking to the prescriber. If something feels like an emergency, call 911.

Written by Kinwise's template (no AI) from 48 evidence records. Knowledge base 2026.10.2 · evidence 1756167f · Reviewed by Alex on Oct 17, 2026, 10:30 AM. Fictional demo data.